



Fiona Stanley Fremantle Hospitals Group

OFFICIAL

Policy and Procedure

Closed Circuit Television Monitoring in Mental Health Clinical Areas

Reference #: FSFH-MENH-POL-0011

Scope

Site	Service/Department/Unit	Disciplines
Fiona Stanley Hospital Fremantle Hospital	Mental Health Service	Medical, Nursing, Allied Health, Facilities Management, SIMS & Security Services

1. Introduction

This policy provides guidance for the responsible use and appropriate management of closed-circuit television (CCTV) for clinical purposes across the mental health services, and to ensure that audio/video monitoring of patient areas complies with legislative and ethical requirements.

CCTV monitoring is one of a number of strategies used to maximise the safety of all patients and staff within mental health service. CCTV is used for visual monitoring and may be used for the purpose of ensuring the safety and welfare of patients and others by enabling a timely and appropriate response in the event of an identified risk to a patient and/or other person. Body Worn Cameras are covered in a separate policy and procedure.

2. Terminology

Closed circuit television (CCTV)	A television system, in which video signals are transmitted from one or more cameras by cable to a set of monitors, used for monitoring the general safety and wellbeing of patients and/or security purposes.
Body Worn cameras (BWC)	Wearable audio, video or photographic recording system. Security Services at both FSH and FH wear BWC which record audio and video when activated. This only occurs if security are responding to an Emergency or a Patient Assist / Standby for an aggressive/high risk patient

3. Policy

- Patients, families and carers will be informed that CCTV monitoring is in use within the mental health service and written information will also be provided including the areas under surveillance.
- Patient privacy and dignity is not to be compromised by CCTV monitoring. Patient confidentiality will be maintained at all times.
- CCTV monitoring will only take place in designated areas, and does not include areas of consumer privacy such as bathrooms and bedrooms.
- CCTV cameras are positioned in entrances, corridors, and areas without direct line of sight, courtyards, outdoor areas and the seclusion rooms/annexe area.
- CCTV footage is information and in certain circumstances, CCTV footage may contain recordings of serious incidents or events. CCTV footage can be used to assist police investigations or be used as evidence in court.
- Video monitoring devices are only for the purpose of visual observation. Video recordings (where available) will not be used for any purpose other than to review and observe safety-related matters.

Audio recordings of patients are expressly prohibited unless embedded in teaching or specialised therapy (e.g. group therapy) locations/inpatient ward and conducted with the patient and their family's written informed consent.

- CCTV monitoring is not a substitute for personal interaction between staff and consumers or a substitute for direct visual observations including face-to-face or via an observation window.
- Monitor screens must not be clearly visible to visitors, members of the public or patients. Only staff with direct clinical responsibility are to undertake monitoring using the screen. Security staff can view and monitor all screens.
- New staff will be notified that CCTV monitoring occurs within the mental health service as part of the orientation process. Staff will be provided with education and training regarding the use of CCTV monitoring to ensure it is used appropriately including legislative and ethical requirements.

Notification signage must be displayed at both the public and police/ambulance entrances to the mental health service to inform visitors that CCTV monitoring is in place. Signs are also placed in areas where CCTV monitoring occurs. These signs are to be large enough to be seen easily and clearly indicate, in writing with a supporting image, that the individual may be monitored.

4. Procedure

4.1. Use of CCTV

- Patient orientation to the mental health service will include where practicable a verbal notification and explanation of CCTV monitoring, staff will document that this has occurred in the medical record.
- Staff will answer any questions consumers or visitors may have about the purpose of monitoring and direct any enquires they are unable to answer to the Clinical Nurse Specialist/Clinical Nurse Manager or Nurse Unit Manager.
- CCTV may be used to monitor a patient in a locked seclusion room but it is not a substitute for the direct visual observations required at least every 15 minutes (s. 222 Mental Health Act 2014).
- Sensitivity and discretion will be used in regard to toileting or maintaining dignity and other personal needs of the consumer in seclusion. The CCTV monitor observation screen may be switched off to allow the consumer privacy after the Shift Coordinator has assessed the consumers risk and if considered clinically appropriate.
- If the CCTV monitor observation screen is switched off direct visual observations will be implemented as determined by the treating team, in addition to the mandated 15 minutely observations.
- When the CCTV monitor observation screen is switched off staff will:
 - Document the reason in the medical record
 - Document the times the monitor was switched off and switched back on again
 - Include details in clinical handover.
 - Be aware the camera is still recording
- CCTV faults are reported to FSH Security Incident Management Service (SIMS) or FH Security

4.2. Requests for CCTV footage

- Requests for CCTV footage will be directed to and actioned by FSH SIMS or FH Security services.

5. Compliance/Performance Monitoring

- The Service 5 Corporate Leadership Committee is responsible for ensuring compliance with this policy and procedure via an annual sexual safety in mental health inpatient units audit.

- The FSFHG Facilities Management and SMHS Security Department will ensure the CCTV system is registered on the Blue Iris CCTV Register operated by WA Police and the Office of Crime Prevention.

6. Related Documents

[Mental Health Act 2014](#)

[Privacy Act 1988](#)

[State Records Act 2000](#)

[Surveillance Devices Act 1998](#)

7. Related Policy Documents

- Department of Health
 - [Department of Health Patient Confidentiality fact sheet](#)
 - [Department of Health Notifiable and Reportable Conduct Policy MP0125/19](#)
- FSFHG
 - Courtyard access and observation

8. Related Standards

- NSQHS Standards
 - Clinical Governance
 - Partnering with consumers
- NSMH Standards:
 - Safety
 - Delivery of Care
- Chief Psychiatrists Standards for Clinical Care
 - Consumer and carer involvement in Individual Care

9. Authorisation

EXECUTIVE SPONSOR: Service Director, Service 5					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
1	08/2019	Nurse Director Service 5	NMPGC	FSFHG Policy Committee	08/2022
2	08/2022	Nurse Director Service 5	SME	FSFHG Policy Committee	08/2025
3	10/2025	CNS -Quality Improvement		SQR Committee Service 5	11/2028